



Report for

TEST: Surgical Pathology
 Collected Date & Time:

Result Name	Results	Units	Reference Range
Surgical Pathology	SURGICAL P		
SURGICAL PATHOLOGY REPORT			

Patient Name:
 Accession #
 Med. Rec. #:
 Billing Type:
 Location:

Service:

Gender: M

Billing

Taken

Physician(s):

Reported:

Specimen(s) Received

- A: Distal right ureter
- B: Distal left ureter
- C: Bladder and prostate
- D: Right pelvic lymph nodes
- E: Left pelvic lymph nodes

1CD-0-3
 carcinoma, urothelial, nos 8120/3
 Site: bladder, nos C67.9

Pathologic Diagnosis

Distal right ureter (A):

- Ureter segment, negative for dysplasia and malignancy.

Distal left ureter (B):

- Benign ureter segment, negative for dysplasia and malignancy.

Bladder and prostate, radical cystoprostatectomy (C):

- Invasive high-grade urothelial carcinoma (4.5 cm) invading through detrusor muscle into perivesical adipose tissue.
- Benign margins of surgical resection.
- Negative for lymphovascular and perineural invasion.
- Lymph nodes (5), no tumor present.
- Prostate with scattered microscopic foci of high-grade prostatic intraepithelial neoplasia; negative for involvement by transitional cell carcinoma.

- Unremarkable seminal vesicles, negative for involvement by urothelial carcinoma.

Right pelvic lymph node (D):

- Lymph node (1), no tumor present.

Left pelvic lymph nodes (E):

- Lymph nodes (4), no tumor present.

SURGICAL PATHOLOGY CANCER CASE SUMMARY CHECKLIST: BLADDER, RADICAL CYSTECTOMY

Specimen: Bladder, prostate, bilateral seminal vesicles

Procedure: Radical cystoprostatectomy

Tumor Site: Left anterior and posterolateral wall

Tumor Size: 4.5 cm

Histologic Type: Urothelial (transitional cell) carcinoma

Associated Epithelial Lesions: Papillary urothelial neoplasm

JW
 11/4/12

high-grade

Histologic Grade: WHO high-grade

Tumor Configuration: Papillary for noninvasive elements and infiltrative for invasive elements

Microscopic Tumor Extension: Perivesical fat

Margins: Uninvolved by invasive carcinoma

Lymph-Vascular Invasion: Not identified

Additional Pathologic Findings: None

PATHOLOGIC STAGE: pT3a, pN0, pMX

NOTE: Information on pathology stage and the operative procedure is transmitted to this Institution's Cancer Registry as required for accreditation by the Commission on Cancer. Pathology stage is based solely upon the current tissue specimen being evaluated and does not incorporate other relevant data. Pathology stage is only a component to be considered in determining the clinical stage and should not be confused with nor substituted for it. The exact operative procedure is available in the surgeon's operative report.

Primary Pathologist:

Electronically Signed Out

Clinical History

Bladder cancer.

Intraoperative Consultation

- A. Distal right ureter: Negative for carcinoma. (Microscopic evaluation).
- B. Distal left ureter: Negative for carcinoma. (Microscopic evaluation).
- D. Right pelvic lymph node: Benign lymph node. (Microscopic evaluation).
- E. Left pelvic lymph node: Negative for carcinoma. (Microscopic evaluation).

Gross Description

- A. Received fresh for frozen section labeled "distal right ureter" is a 0.5 x 0.3 x 0.1 cm segment of ureter. The specimen is inked yellow and entirely submitted in cassette A1-FS.
 - B. Received fresh for frozen section labeled "distal left ureter" is a 0.5 x 0.4 x 0.1 cm segment of ureter. The specimen is inked black and entirely submitted in cassette B1-FS.
 - C. Received in formalin and labeled "bladder and prostate" is a 469 g, cystoprostatectomy specimen (15.0 x 13.0 x 6.0 cm). The specimen comes with an abundant amount of perivesicular adipose tissue attached. The cystoprostatectomy specimen consists of a bladder (6.5 x 5.5 x 5.0 cm) with attached right and left ureter (1.0 cm in length). There is a prostate (5.0 cm from left-to-right, 3.5 cm from anterior-to-posterior, 3.3 cm from apex-to-bladder-neck-margin) with attached right seminal vesicle (3.0 x 1.5 x 0.5 cm), right vas deferens (12.0 x 0.5 x 0.2 cm), left seminal vesicle (3.5 x 2.5 x 0.6 cm), left vas deferens (3.0 x 0.3 x 0.2 cm). The right side of the specimen is inked blue; the left side of the specimen is inked black.
- In the left anterior and posterior lateral wall of the bladder, there is a 4.5 x 2.5 x 0.9 cm pink-tan polypoid mass. This lesion extends across the midline into the right anterior and posterior bladder and extends towards the bladder-prostate junction. The

mass' cut surface is white-tan, firm and extends into the perivesicular fat. The mass at the posterior bladder, comes to within 0.5 cm to the perivesicular fat margin (inked black) and on the anterior bladder is 1.0 cm from the perivesicular fat margin (inked black). The lesion is 2.5 cm from the nearest peritonealized surface. The ureteral orifices are patent. There are no other discrete masses or suspicious areas grossly identified. The remainder of the mucosa is tan, glistening and pliable.

The prostate has a white-tan cut surface with no discrete nodules or masses grossly identified. There are 5 lymph nodes dissected out ranging from 0.3 to 1.1 cm in greatest dimension. Also received in the container is a 38 cm in length and 0.8 cm in diameter, brown-tan rubber tubing consistent with a catheter. Sections are submitted as follows: C1 - right trigone, C2 - left trigone, C3 - posterior bladder-prostate transition, C4 - anterior bladder-prostate transition, C5-9 - mass in left lateral posterior wall, C10 - mass and lateral anterior wall, C11 - right lateral wall, C12 - posterior wall, C13 - anterior wall, C14 - dome, C15 - right seminal vesicle insertion point and cross-sections through right seminal vesicle and vas deferens, C16 - left seminal vesicle insertion point and cross-section through left seminal vesicle and vas deferens, C17-18 - right anterior apex perpendicularly cut, C19-20 - left apex perpendicularly cut, C21 - right anterior prostate, C22 - left anterior prostate, C23-25 - right posterior prostate, C26-28 - left posterior prostate, C29 - 5 lymph nodes ranging from 0.3 to 1.1 cm in greatest dimension.

D. Received fresh for frozen section labeled "right pelvic lymph node" is a single lymph node measuring 4.0 cm in greatest dimension. The lymph node is serially sectioned and entirely submitted in cassette D1-FS and D2-FS.

E. Received fresh for frozen section labeled "left pelvic lymph nodes" are 4 lymph nodes ranging from 1.3 to 5.5 cm in greatest dimension. Sections are submitted as follows: E1-FS and E2-FS contains one lymph node (5.5 cm) inked blue and serially sectioned, E3-FS - two lymph nodes, one inked yellow, one inked black, E4-FS and E5-FS - one lymph node (5.0 cm) inked orange and bisected.

: [REDACTED]

Microscopic Description

Microscopic examination has been performed on all slides. The pathologic diagnosis encompasses the essential microscopic findings of this case.

Interpretation performed at [REDACTED]

If immunohistochemistry or in situ hybridization was used the following applies: This test [REDACTED] characteristic determined by [REDACTED]. It has not been cleared or approved by the U. S. Food and Drug Administration (FDA). The FDA does not require this test to go through premarket FDA review. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments (CLIA) as qualified to perform high complexity clinical laboratory testing. All special stains

[REDACTED]

used have adequate controls.

Criteria	Yes	No
Diagnostic Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary		☒
Case is (circle):	QUALIFIED / DISQUALIFIED	
Reviewed by: <i>[Signature]</i>	Date Reviewed: 11/2/12	